

Visit Number:	42426501357	Registered	04-02-2026 10:40:23
Patient Name:	محمود علاء الدين محمد	Collected	04-02-2026 10:40:45
Age / Sex:	22 / Male	Authenticated	05-02-2026 16:28:14
Referred By:	طلعت عبد العاطي : Prof	Printed	10-09-2026 15:10:04
Client Name:	8408		

TEST NAME

RESULT

REFERENCE RANGE

Molecular Genetics

Familial Mediterranean Fever (MEFV Gene) Screening

Result:	Positive heterozygous at codon P369S & M694I	Not Detected
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N.B. : Sample type: Whole Blood, EDTA

Background

Characteristics: Familial Mediterranean Fever (FMF) is a genetic condition characterized by recurrent but short lived attacks of fever, abdominal pain, joint pain and or skin rashes. Symptoms and frequency of these attacks are highly variable. Renal amyloidosis is another common complication in untreated individuals and may be the only manifestation in some patients.

Cause: Pathogenic germline variants in MEFV gene.

Inheritance: Autosomal inherited genetic autoinflammatory disorder. Five founder mutations, V726A, M694V, M694I, M680I and E148Q account for 74% of FMF chromosomes from typical cases (Armenians, Arabs, Jews, and Turks). Rare mutations are preferentially found in populations not usually affected by FMF (eg Europeans not from the above ancestries). The various combinations of MEFV mutations define severe to mild genotypes. Carrier rates for FMF mutations may be as high as 1:3 in some populations, suggesting that the disease is underdiagnosed.

Method

This assay uses polymerase chain reaction (PCR) and reverse-hybridization for the identification of 12 mutations in the MEFV gene, namely: E148Q, P369S, F479L, M680I (G/C), M680I (G/A), I692del, M694V, M694I, K695R, V726A, A744S, R761H

Limitations

MEFV genetic defects not covered by the assay cannot be excluded. Results of genetic screening should always be correlated with clinical presentation and family history. If concern for a periodic fever remains, gene sequencing is advised

Recommendations

For positive cases : Genetic consultation is indicated, including a discussion of medical screening and management. At-risk family members should be offered testing for the identified pathogenic MEFV variant. This individual's reproductive partner should be offered carrier screening for FMF.



Scan Me

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